

ADHD in Women - Printable Sheets

From *ADHD in Women* by Ruth Callahan (Oro Enigma Publishing). Print as many copies as you need.

None of these sheets is a diagnostic test. They are for recognising patterns and for taking evidence to a clinician, which is the thing that makes an appointment useful.

THE RECOGNITION CHECKLIST

Tick what applies, and - this is the part that matters - **write your own example beside it**. A specific example with a consequence is worth more to an assessor than a hundred ticks.

	Trait	My example, and what it cost
	Losing the thread mid-page or mid-sentence	
	No internal sense of how long things take	
	Walking into rooms and forgetting why	
	Starting enthusiastically, not finishing	
	The admin pile - unopened, undone	
	Hyperfocus on the wrong thing	
	A mind that will not stop	
	Restlessness that looks like anxiety	
	Talking a lot, then replaying it mortified	
	Emotional reactions out of scale, slow to settle	
	Sensory overload - noise, labels, lights	

	Exhaustion not explained by the workload	
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The masking questions - these are the ones nobody asks:

		My example
	Systems on top of systems to appear organised	
	Over-preparing, rehearsing, arriving absurdly early	
	Working twice as long for the same result	
	Choosing work or a life that hides it	
	Perfectionism as cover	

THE ASSESSMENT PREPARATION SHEET

Six to ten specific examples, with consequences

Not "I'm disorganised" - *"I missed my own mortgage renewal twice and it cost about £900."*

#	What happened	What it cost
1		
2		
3		
4		
5		
6		
7		
8		

Childhood evidence

Source	Have it?	Notes
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School reports		
Parent / sibling who can answer questions		
My own specific memories before age 13		
Diaries, letters, anything written		

Phrases from my reports:

Medical history to bring

	Detail
Mental health diagnoses, when, how well treated	
Current and past medication	
Thyroid / ferritin / B12 / vitamin D results	
Sleep	
Alcohol and substance use (honestly - it changes what they prescribe)	
Cardiac history, mine and family	

Does my cycle track my symptoms?	
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My three questions, written on a card

1. _____
2. _____
3. _____

Remember: describe the bad days, not the managed ones - and say what the managed ones cost. Masking switches on automatically in front of a professional.

THE CYCLE AND SYMPTOMS TRACKER

Three months. Score each out of 5. Mark day 1 of your period in the left column.

Month 1 - period started: ____ / ____

Day	Focus /5	Mood /5	Reactivity /5	Notes
1-3				
4-7				
8-11				
12-15				
16-19				
20-23				
24-28				

Month 2 - period started: ____ / ____

Day	Focus /5	Mood /5	Reactivity /5	Notes
1-3				
4-7				
8-11				

12-15				
16-19				
20-23				
24-28				

Month 3 - period started: ____ / ____

Day	Focus /5	Mood /5	Reactivity /5	Notes
1-3				
4-7				
8-11				
12-15				
16-19				
20-23				
24-28				

My best week of the month: _____ **My worst:**

Decisions I will not make in the worst week:

Take this to your prescriber. "My medication works less well in the week before my period" is a specific, actionable observation.

THE DOCTOR'S APPOINTMENT

SCRIPT

The sentence that connects both halves (Chapter 10):

"My symptoms follow my cycle - reliably worse in the week before my period - and over the last ____ years they have got worse overall as my periods have become irregular. I think the hormonal change and the attention problems are connected, and I would like both looked at."

Questions worth asking - tick the ones you need:

- ☐ Could this be perimenopause, ADHD, or both?
- ☐ Is HRT appropriate for me, and what would it be for?
- ☐ Should my ADHD medication be reviewed as my hormones change?
- ☐ Should the premenstrual week be managed differently?
- ☐ Could this be PMDD?
- ☐ What else should be excluded - thyroid, ferritin, B12, sleep?

If I am dismissed, what I will say:

"I'd like a second opinion" / "Could I be referred to a specialist service?"

Who is coming with me: _____

THE MEDICATION DIARY - FIRST SIX WEEKS

Recall of a six-week period is exactly the thing you are not good at. Write it down.

Week	Medicine + dose	Time taken	What changed (good)	Side effects	Sleep	Appetite
1						
2						
3						
4						
5						
6						

The afternoon drop - when does it happen? _____

Does it work differently in my premenstrual week?

The goal is not the highest tolerable dose. It is the dose where function improves and side effects are acceptable.

THE WORKPLACE ADJUSTMENTS REQUEST

Adapt and send. Frame around output, be specific, ask for a time-limited trial - much easier to agree to, and it usually becomes permanent.

Dear _____,

I wanted to put in writing a few working arrangements that I think would let me do my best work. I'd like to suggest we trial them for a month and then review.

- 1. _____
- 2. _____
- 3. _____

I'm happy to discuss any of these.

Pick from:

	Adjustment
	Noise-cancelling headphones, with permission to wear them
	A quiet space, or home-working days for focused work
	Written confirmation after verbal instructions
	Agendas in advance, minutes afterwards

	Flexible start time
	Large tasks staged, with interim check-ins
	Admin batched into one supported block
	One manager, one channel for instructions

THE INVISIBLE LOAD INVENTORY

Chapter 17. Write down **everything**, including the parts nobody counts. Most couples have never seen this on paper.

The task	The invisible part (noticing, planning, remembering)	Who does it now	Who could own the whole domain
Food	Knowing what is needed, planning, list		
Laundry	Noticing what is needed and when		
School / childcare	Dates, forms, kit, parties, gifts		
Health	Appointments, prescriptions, follow-ups		
Money	Bills, renewals, deadlines		
House	Repairs, maintenance, noticing		
Family	Birthdays, cards, calls, visits		

Domains I am handing over completely (not tasks - domains):

1. _____

2. _____

"Can you do the shopping" leaves you the planning. "You own food - planning, buying, cooking, Monday to Wednesday" removes it.

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